This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Doxycycline or Azithromycin for the treatment of Chlamydia

Patient Group Direction, version 005, issued 30 September 2026. Supersedes Version 004, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Chlamydia

Overview

- **Chlamydia trachomatis** is the most common **bacterial sexually transmitted infection (STI)** in the UK.
- Often **asymptomatic** (up to 70% of women and 50% of men).
- Untreated infection can lead to complications: **pelvic inflammatory disease (PID)**, **infertility**, **ectopic pregnancy**, and **epididymo-orchitis**.

Assessment

Symptoms (when present):

- **Women**: vaginal discharge, intermenstrual/postcoital bleeding, dysuria, pelvic pain.
- **Men**: urethral discharge, dysuria, testicular pain (epididymo-orchitis).
- **Both**: rectal pain/discharge (if rectal infection), conjunctivitis (rare).

Risk factors:

- Age under 25.
- Multiple sexual partners.
- New sexual partner in the last 12 months.
- Inconsistent condom use.

Testing

- Use **NAAT (nucleic acid amplification test)** – highly sensitive.
  - **Women**: self-taken vulvovaginal swab (preferred), or endocervical swab.
  - **Men**: first-catch urine sample.
- Offer **rectal/pharyngeal swabs** based on sexual practices.

Management

First-Line Treatment

- **Doxycycline 100 mg twice daily for 7 days**
  - Effective for urogenital, rectal, and oropharyngeal chlamydia.
  - Avoid in pregnancy.

Alternatives (e.g. pregnancy):

- **Azithromycin 1 g on day 1, then 500 mg once daily on days 2 and 3 (BASHH 2015 chlamydia guideline, 2018 update: the single 1 g dose is no longer recommended because of treatment failure and Mycoplasma genitalium resistance).**
- **Erythromycin** or **amoxicillin** may be used if doxycycline and azithromycin unsuitable.

Partner Notification and Treatment

- Notify and treat **all sexual partners** from the **preceding 6 months**.
- Partners should be tested and **empirically treated** without waiting for results.
- Advise **no sex until both patient and partner(s) have completed treatment**.

Follow-Up

- **Test of cure**:
  - Not routinely needed unless:
    - Pregnancy
    - Non-standard treatment
    - Persistent symptoms
    - Rectal infection
  - If required, do at least **3 weeks after treatment** to avoid false positives.
- **Retesting recommended after 3 months** to detect reinfection, especially in under-25s.

Prevention and Education

- Promote **condom use**.
- Offer **full STI screening**, including HIV and syphilis.
- Offer **contraceptive advice** as needed.
- Under-25s should be screened **annually** or with each new partner (as per National Chlamydia Screening Programme).

Patient Group Direction

for the supply of

Doxycycline 100mg capsules

for the treatment of

Chlamydia

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Doxycycline is indicated for the treatment of uncomplicated genital chlamydia trachomatis infection in individuals aged 16 years and over, or aged 13 to 15 where Fraser competence and a safeguarding assessment are recorded, as set out in the inclusion criteria.
Inclusion criteria	- Individuals aged 16 years and over; or aged 13 to 15 where the pharmacist has assessed and recorded Fraser competence and completed a safeguarding assessment (partner age, coercion, exploitation indicators) with no concern. - Confirmed or strongly suspected diagnosis of genital chlamydia infection. - Informed consent obtained. - No contraindications to doxycycline.
Exclusion criteria	- Aged under 13: any sexual activity is a safeguarding concern; do not supply, refer to the GP or sexual health service the same day and make a safeguarding referral. - Aged 13 to 15 where Fraser competence is not established, or any safeguarding concern (partner 18 or over, coercion, exploitation, learning disability): refer and follow the local safeguarding pathway. - Known hypersensitivity to tetracyclines. - Pregnant or breastfeeding individuals. - Severe hepatic insufficiency. - Known or suspected complicated infection (e.g., PID). - Inability to comply with 7-day regimen or swallow tablets.
Cautions	- Advise on taking with water and remaining upright for 30 minutes to avoid oesophageal irritation. - Avoid sun exposure during and after treatment due to risk of photosensitivity. - Counsel on abstaining from sexual activity until treatment and partner treatment are completed. - Use effective contraception during and for 7 days after completing the course.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Doxycycline 100mg capsules
Legal category	POM
Storage	Store below 25°C. Protect from moisture and light.
Route/method of administration	Oral administration with a full glass of water while sitting or standing.

Dose and frequency	100 mg twice daily for 7 days.
Quantity to be administered and/or supplied	14 capsules.
Maximum or minimum treatment period	7 days continuous course.
Adverse effects	Nausea, vomiting, diarrhoea, abdominal pain, photosensitivity, oesophageal irritation. Rare: hypersensitivity reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Do not have sex, including oral sex and sex with a condom, until 7 days after you and your partner(s) have completed treatment. Your sexual partners from the last 6 months need to be tested and treated. Seek medical advice if symptoms worsen, have not settled within 2 weeks of finishing treatment, or you become systemically unwell. A test of cure is needed at least 3 weeks after treatment if you are pregnant, had rectal infection, had non-standard treatment or symptoms persist. Retest for reinfection at 3 months.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	25th June 2025

Patient Group Direction

for the supply of

Azithromycin 500mg tablets

for the treatment of

Chlamydia

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Azithromycin is indicated for the treatment of uncomplicated genital chlamydia trachomatis infection in individuals aged 16 years and over, or aged 13 to 15 where Fraser competence and a safeguarding assessment are recorded, as set out in the inclusion criteria, where doxycycline is unsuitable or contraindicated.
Inclusion criteria	- Individuals aged 16 years and over; or aged 13 to 15 where the pharmacist has assessed and recorded Fraser competence and completed a safeguarding assessment (partner age, coercion, exploitation indicators) with no concern. - Confirmed or strongly suspected diagnosis of genital chlamydia infection. - Informed consent obtained. - Doxycycline unsuitable or contraindicated.
Exclusion criteria	- Aged under 13: any sexual activity is a safeguarding concern; do not supply, refer to the GP or sexual health service the same day and make a safeguarding referral. - Aged 13 to 15 where Fraser competence is not established, or any safeguarding concern (partner 18 or over, coercion, exploitation, learning disability): refer and follow the local safeguarding pathway. - Known hypersensitivity to macrolides. - History of QT prolongation or taking interacting QT-prolonging drugs. - Severe hepatic impairment. - Pregnant or breastfeeding: refer to the GP or sexual health service (azithromycin is the BASHH choice in pregnancy, but a test of cure and follow-up are needed). - Concurrent use of ergot derivatives.
Cautions	- Use with caution in patients with mild to moderate liver or kidney impairment. - Advise patient not to take antacids 2 hours before or after dose. - Counsel on abstaining from sexual activity for 7 days after treatment and until partners are treated. - Reinforce need for test of cure if symptoms persist or in pregnancy.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Azithromycin 500mg tablets
Legal category	POM
Storage	Store below 25°C. Protect from moisture and light.

Route/method of administration	Oral administration with or without food.
Dose and frequency	1 g (two 500 mg tablets) on day 1, then 500 mg (one tablet) once daily on days 2 and 3. Total 2 g over 3 days.
Quantity to be administered and/or supplied	4 x 500 mg tablets (total 2 g).
Maximum or minimum treatment period	3 days.
Adverse effects	Nausea, vomiting, diarrhoea, abdominal pain, flatulence, headache. Rare: QT prolongation or allergic reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Do not have sex, including oral sex and sex with a condom, until 7 days after you and your partner(s) have completed treatment. Your sexual partners from the last 6 months need to be tested and treated. Seek medical advice if symptoms worsen, have not settled within 2 weeks of finishing treatment, or you become systemically unwell. A test of cure is needed at least 3 weeks after treatment if you are pregnant, had rectal infection, had non-standard treatment or symptoms persist. Retest for reinfection at 3 months.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance
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Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	25th June 2025

Version and change record

Version: v005

Issued: 30 September 2026

Supersedes: Version 004, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v004

Issued: 14 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Indication rows in both arms updated to 16 and over, or 13 to 15 with recorded Fraser competence and safeguarding assessment; they still said 15 and over after the safeguarding change.

Cover and arm headings corrected from administration to supply.

Doxycycline quantity states 14 capsules; the product is capsules. Azithromycin named as 500mg tablets.

Follow-up advice replaced with chlamydia-specific advice: abstinence for 7 days after both partners are treated, partner notification, test of cure at 3 weeks where indicated, and retest at 3 months.

Broken NICE MPG2 link corrected.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v002

Issued: 11 September 2026

Supersedes: Version 001, 25 June 2025

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

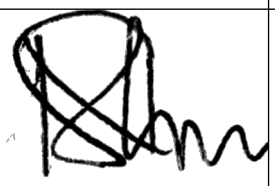
Safeguarding written into both arms: under 13 never supplied and always referred with a safeguarding referral; 13 to 15 only with recorded Fraser competence and a safeguarding assessment; the previous version included everyone aged 15 and over with no assessment.

Azithromycin regimen corrected to the BASHH regimen (1 g then 500 mg daily for 2 days, 4 tablets, 3 days). The single 1 g dose in the previous version has not been recommended since 2018. The 'unless assessed as suitable by a prescriber' pregnancy line is replaced by a referral.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v005, 30 September 2026.

Name	***Qualification***	***Signature***	***Date***
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.